7.1 Urinary Tract Infection*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "It burns when I pass urine and I go very often." **Objective** • Cloudy, foul-smelling urine • Dipstick positive for nitrites and leukocytes • Suprapubic tenderness	**ACTUAL** Impaired urinary elimination related to bladder infection as evidenced by dysuria, frequency and urgency.	• Burning and frequency gone within 48–72 hours of treatment.	• Collect a midstream specimen before antibiotics, as ordered. • Give antibiotics as prescribed and stress finishing the course. • Encourage 2–3 L of fluid a day unless restricted. • Encourage her to void often and empty the bladder fully.	• Met when symptoms resolve.
Subjective • Rates suprapubic pain 5/10 **Objective** • Suprapubic tenderness • Discomfort on voiding	**ACTUAL** Acute pain related to bladder mucosal inflammation as evidenced by suprapubic pain and burning.	• Pain at or below 3/10 within 24 hours.	• Give analgesia or a urinary analgesic as prescribed. • Offer warmth to the lower abdomen if it soothes.	• Achieved when pain is controlled.
Subjective • "I get this infection every few months." **Objective** • Recurrent UTIs • Sexually active woman	**RISK** Risk for infection [recurrence] as evidenced by risk factors: history of recurrent urinary tract infection.	• Describes how to prevent further episodes.	• Teach wiping from front to back. • Advise passing urine after intercourse. • Encourage enough fluid and not holding urine for long.	• Outcome reached when she describes the preventive measures.

* **References for this care plan:** [32], [35], [37], [48]. Full entries are listed in the References section of the CPMN book.